REASON FOR CONSULTATION: Evaluation of chronic asymptomatic alanine aminotransferase (ALT) and aspartate aminotransferase (AST) elevation discovered during routine executive physical examination.

HISTORY OF PRESENT ILLNESS: The patient is a 51-year-old male with a medical history notable for type 2 diabetes mellitus, metabolic syndrome, arterial hypertension, and class II obesity (BMI 36.4 kg/m2). He reports no abdominal pain, jaundice, pruritus, lower extremity edema, easy bruising, or cognitive changes. Alcohol consumption is negligible (<1 standard alcoholic beverage per month). He takes metformin, lisinopril, and rosuvastatin.

PHYSICAL EXAMINATION:

Vital Signs: BP 136/84 mmHg, HR 72 bpm, SpO2 98%, BMI 36.4 kg/m2 (Weight 112 kg, Height 175 cm).

Abdomen: Protuberant, soft, non-tender throughout. Liver edge is smooth and palpated 3 cm below the right costal margin with a total vertical span of 14 cm to percussion (hepatomegaly). No splenomegaly, shifting dullness, or abdominal wall collateral veins (caput medusae).

Skin: No spider angiomas, palmar erythema, or scleral icterus. Acanthosis nigricans present along the posterior neck and axillary folds.

DIAGNOSTIC WORKUP:

Laboratory Evaluation: AST 64 U/L (normal < 35), ALT 88 U/L (normal < 45), Alkaline Phosphatase 98 U/L, Total Bilirubin 0.8 mg/dL, Albumin 4.3 g/dL, Platelet count 245,000/uL, Hemoglobin A1c 7.8%, Fasting Triglycerides 285 mg/dL.

Secondary Liver Disease Exclusions: Hepatitis B surface antigen non-reactive, Hepatitis C antibody non-reactive, Antinuclear antibody (ANA) negative, Anti-smooth muscle antibody (ASMA) negative, Serum ferritin 310 ng/mL with transferrin saturation 28% (excludes hemochromatosis), Ceruloplasmin and Alpha-1 antitrypsin levels normal.

FIB-4 Index: Calculated at 1.48 (intermediate risk for advanced fibrosis).

Vibration-Controlled Transient Elastography (FibroScan): Controlled Attenuation Parameter (CAP) is 342 dB/m, consistent with severe grade 3 hepatic steatosis (>66% hepatocytes). Liver stiffness measurement is 8.6 kPa, corresponding to stage F2 moderate liver fibrosis.

IMPRESSION AND CLINICAL MANAGEMENT:

1. Non-Alcoholic Steatohepatitis (NASH / Metabolic Dysfunction-Associated Steatohepatitis - MASH) with stage 2 fibrosis.

2. Intensify metabolic control: Refer to clinical nutrition for Mediterranean hypocaloric dietary modification with a target weight loss of 7-10% of total body weight.

3. Consider initiating a GLP-1 receptor agonist (e.g., semaglutide) for combined glycemic control, weight reduction, and histological resolution of steatohepatitis.

4. Repeat liver elastography and transaminases in 12 months.